Cervical Screening & Bimanual Pelvic Examination — OSCE Checklist

Deakin MD2 · ELC8 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Prepare and consent → Equipment → Position → Inspect → Speculum + CST → Bimanual → Close**. *Practised on part-task trainers — speak to the model as you would to a patient.*

1. Before the patient is on the couch

☐	Why a pelvic examination is needed; what it involves (speculum, CST, swabs, bimanual) and the position
☐	Past experience of pelvic examinations
☐	Obstetric history: children? vaginal births? (guides speculum size)
☐	Menstrual history: LMP; possibility of pregnancy
☐	Verbal consent; "Just say stop and I will"
☐	Offer a chaperone (advocate for patient and doctor; often a nurse who assists)
☐	Offer to empty bladder first
☐	Timing*: CST ideally ~2 weeks after day 1 of last period (if menstruating)

2. Equipment

☐	Gloves	Cushion / folded blanket to raise pelvis	Manoeuvrable light
☐	Lubricant	Range of speculum sizes	Swabs if needed
☐	Cytobrush + liquid-based vial (ThinPrep)	Single-use mat (bluey)	Sanitary pad and tissues
☐	Chaperone present	Vial labelled and form completed †	

3. Position and inspection

☐	Gather equipment; hand hygiene; gloves
☐	Patient undresses from waist down behind curtain and covers with blanket; enter with permission
☐	Position: supine, knees drawn up, heels together, let knees fall apart (do **not** push knees apart)
☐	Expose waist down only when ready; adjust light
☐	Inspect external genitalia: vulval skin, discharge, lesions, prolapse †



Position: heels together, knees fall apart

4. Speculum and cervical screening test

☐	Warn the patient; part labia with non-dominant hand
☐	Insert lubricated speculum with blades sideways, rotating during insertion as patient breathes out; aim towards sacrum, not head
☐	Gently open blades until cervix is visualised
☐	Fix speculum open; inspect cervix: os, ectropion, polyps, discharge, lesions †
☐	Insert cytobrush into the os and rotate 2–3 times
☐	Remove brush and agitate well in the ThinPrep liquid
☐	Loosen speculum and remove gently as patient breathes out (watch vaginal walls †)

5. Bimanual examination (if indicated)

☐	Warn patient; gently touch the introitus, then slowly insert 2 lubricated gloved fingers
☐	Palpate cervix: position, consistency, os, tenderness †
☐	**Cervical excitation**: gently move the cervix side to side — watch the face for pain
☐	Uterus: other hand on lower abdomen pressing down towards internal fingers — size, shape, position (anteverted / retroverted), mobility, tenderness †
☐	Adnexa: internal fingers in each lateral fornix, abdominal hand over iliac fossa — masses, tenderness; both sides
☐	Withdraw fingers; check glove for blood or discharge

6. Closing

☐	Cover patient; tell them the examination is complete; leave tissues and bin; privacy to dress
☐	Explain findings and how/when CST results will be received; follow-up plan
☐	Invite questions; check comfort
☐	Document consent, chaperone, findings, tests taken, follow-up

7. Quick reference

Bimanual finding †	Think of
Cervical excitation (motion tenderness)	Pelvic inflammatory disease, ectopic pregnancy
Enlarged, irregular, firm uterus	Fibroids; pregnancy (smooth, soft)
Bulky, tender uterus	Adenomyosis
Fixed, retroverted, tender uterus ± nodules	Endometriosis
Adnexal mass	Ovarian cyst or tumour, ectopic, tubo-ovarian abscess

- Bimanual examination is **not** recommended routinely in asymptomatic people attending for screening; it's for symptoms and pregnancy.
- Australian NCSP: HPV-based CST every 5 years for people with a cervix aged 25–74; self-collection is an option for everyone †.
- Clinician collection (HPV + liquid-based cytology) is required for symptoms (PCB, IMB, PMB, persistent unexplained discharge) and for AIS surveillance.
- Pitfalls: blades inserted vertically or aimed upwards, closing blades on the cervix, no chaperone offer, not warning before touching, not watching the face.